

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of MMRVaxPRO or Priorix for vaccination against Measles, Mumps and Rubella (MMR)

Patient Group Direction, version 004, issued 11 September 2026. Supersedes Version 003, 10 September 2026.

Summary of the governing guidance for MMR vaccination: Green Book chapters 21 (Measles), 23 (Mumps) and 28 (Rubella), the UK routine immunisation schedule and the MMRvaxPro and Priorix SmPCs. NICE CKS 'Immunizations - childhood' summarises the same schedule.

Overview

- The **MMR vaccine** is a **live attenuated vaccine** providing protection against:
 - **Measles** – highly contagious, risk of serious complications (e.g. encephalitis, pneumonia).
 - **Mumps** – can cause meningitis, orchitis, and hearing loss.
 - **Rubella** – especially dangerous in pregnancy, causing congenital rubella syndrome.

Routine Immunisation Schedule (UK)

Age	Dose
12–13 months	First dose of MMR
3 years 4 months (pre-school)	Second dose of MMR

- Can be given **from 6 months** in high-risk situations (e.g. outbreaks or travel to endemic areas), but this dose does **not count toward the routine schedule**.

Catch-Up Vaccination

- Recommended for **anyone born after 1970** who has not had **2 documented doses**.
 - Offer opportunistically (e.g. during GP or travel clinic visits).
 - Ensure a **4-week interval** between doses.
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Eligibility

- Children as part of the routine programme.
 - **Unvaccinated adults** or those with incomplete vaccination history.
 - **Healthcare workers** without evidence of immunity.
 - **Students, travelers, and close contacts** during outbreaks.
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Contraindications

- **Severe immunosuppression**
- **Pregnancy** (avoid conception for **1 month** after vaccination)
- History of **anaphylaxis to neomycin** or gelatin (if vaccine contains it)

Note: Egg allergy is *not* a contraindication.

Precautions

- Postpone in **acute febrile illness** (minor infections are not a reason to delay).
 - Consider specialist advice for those with HIV or immunodeficiency.
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Side Effects

- Usually mild and self-limiting:
 - Fever, rash (5–10 days after), malaise
 - Mild parotid swelling (mumps-like)
 - Transient arthralgia (more common in adult women)
 - Rare: febrile convulsions, thrombocytopenia, anaphylaxis
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Effectiveness

- Two doses provide:
 - **>99% protection** against **measles and rubella**
 - **88–95% protection** against **mumps**

Documentation and Reporting

- Record **vaccine name, batch number, site, route, and date.**
- Report adverse events to the **Yellow Card Scheme.**

Patient Group Direction

for the supply/administration of

MMRVaxPRO 5mg

for vaccination against

Measles, Mumps and Rubella (MMR)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	MMRVaxPRO is indicated for active immunisation against measles, mumps and rubella in individuals from 12 months of age.
Inclusion criteria	- Individuals aged 12 months and over without two documented doses of MMR (catch-up, healthcare workers, students, travellers), or where protection is otherwise required. - Children eligible for the NHS childhood programme must be told they can be vaccinated free by their GP before any private supply; record that this was done. - Informed consent obtained, from a person with parental responsibility or from the young person where Gillick competent. - No contraindications to MMR vaccination.
Exclusion criteria	- Known hypersensitivity to neomycin, gelatin, or any other component of the vaccine, or anaphylaxis to a previous measles, mumps or rubella containing vaccine. - Immunocompromised individuals or those receiving immunosuppressive therapy. - Blood dyscrasias, leukaemia, lymphoma or other malignant neoplasm of the haematopoietic or lymphatic system. - Family history of congenital or hereditary immunodeficiency, unless immune competence has been demonstrated. - Active untreated tuberculosis. - Pregnancy or planning pregnancy within one month. - Yellow fever or varicella vaccine within the previous 4 weeks: defer. Never give yellow fever vaccine and MMR on the same day. - Acute febrile illness (vaccination should be postponed).
Cautions	- Blood products or immunoglobulin in the previous 3 months: defer where possible; where protection is needed now, give and repeat after 3 months (Green Book). Record which applied. - Use caution in children with a history of thrombocytopenia or febrile seizures. - Observe the patient for 15 minutes post-vaccination. - Provide information on common side effects and when to seek further medical advice.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	MMRVaxPRO
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Subcutaneous injection, preferably in the upper arm or thigh.
Dose and frequency	Two doses of 0.5 mL, at least 4 weeks apart (at least 3 months apart where both doses are given under 18 months of age). Doses given before the first birthday do not count towards the course.
Quantity to be administered and/or supplied	0.5 mL per dose.

Maximum or minimum treatment period	Two doses, usually given at age 12–13 months and a second dose before school entry.
Adverse effects	Fever, rash, irritability, swelling or pain at the injection site. Rare: febrile convulsions, thrombocytopenia, allergic reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. Record also the vaccine brand, batch number, expiry date, anatomical site, dose number (1 or 2) and the date the next dose is due. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapters 21, 23 and 28; UKHSA, the complete routine immunisation schedule, current edition; MMRvaxPro and Priorix Summaries of Product Characteristics, current versions

- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
11/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date



Name of professional group
signatory

Job title & name of
organisation

Signature

Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025

Patient Group Direction

for the supply/administration of

Priorix



for vaccination against

Measles, Mumps and Rubella (MMR)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Priorix is indicated for active immunisation against measles, mumps and rubella in individuals from 12 months of age.
Inclusion criteria	- Individuals aged 12 months and over without two documented doses of MMR (catch-up, healthcare workers, students, travellers), or where protection is otherwise required. - Children eligible for the NHS childhood programme must be told they can be vaccinated free by their GP before any private supply; record that this was done. - Informed consent obtained, from a person with parental responsibility or from the young person where Gillick competent. - No contraindications to MMR vaccination.
Exclusion criteria	- Known hypersensitivity to neomycin, gelatin, or any other component of the vaccine, or anaphylaxis to a previous measles, mumps or rubella containing vaccine. - Immunocompromised individuals or those receiving immunosuppressive therapy. - Blood dyscrasias, leukaemia, lymphoma or other malignant neoplasm of the haematopoietic or lymphatic system. - Family history of congenital or hereditary immunodeficiency, unless immune competence has been demonstrated. - Active untreated tuberculosis. - Pregnancy or planning pregnancy within one month. - Yellow fever or varicella vaccine within the previous 4 weeks: defer. Never give yellow fever vaccine and MMR on the same day. - Acute febrile illness (vaccination should be postponed).
Cautions	- Blood products or immunoglobulin in the previous 3 months: defer where possible; where protection is needed now, give and repeat after 3 months (Green Book). Record which applied. - Use caution in children with a history of thrombocytopenia or febrile seizures. - Observe the patient for 15 minutes post-vaccination. - Provide information on common side effects and when to seek further medical advice.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Priorix
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Subcutaneous injection, preferably in the upper arm or thigh.
Dose and frequency	Two doses of 0.5 mL, at least 4 weeks apart (at least 3 months apart where both doses are given under 18 months of age). Doses given before the first birthday do not count towards the course.
Quantity to be administered and/or supplied	0.5 mL per dose.

Maximum or minimum treatment period	Two doses, usually given at age 12–13 months and a second dose before school entry.
Adverse effects	Redness, swelling and pain at injection site, fever, rash. Rare: febrile convulsions, allergic reactions. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. Record also the vaccine brand, batch number, expiry date, anatomical site, dose number (1 or 2) and the date the next dose is due. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history

Version number	Change details	Date
001	Development & issue of new PGD	25 th June 2025



Vaccine safety requirements

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

Anaphylaxis: what must be in place before you vaccinate

ADRENALINE (EPINEPHRINE) 1 IN 1,000 INJECTION must be immediately available in the room where vaccination takes place, in date, together with a telephone.

A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training.

Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint after an injection is common; anaphylaxis is very rare.

Report any suspected anaphylaxis via the Yellow Card Scheme even where the diagnosis is uncertain.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be **QUARANTINED** and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.

**Disposal**

Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.

Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.

Never re-sheath a needle.

Consent in children and young people

Where the individual is UNDER 16, valid consent must come from a person with PARENTAL RESPONSIBILITY, or from the young person where you assess them as GILICK COMPETENT.

Record which of the two applied. Where consent was given by a person with parental responsibility, record their name and relationship to the patient. Where the young person consented on the basis of Gillick competence, record the basis of that assessment.

A parent accompanying a child does not automatically hold parental responsibility. Ask. Users must be competent in assessing consent in children and young people before working under this PGD.

Version and change record

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references name the Green Book chapters, the routine schedule and the SmPCs as the governing sources.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Anaphylaxis: what must be in place before you vaccinate, Cold chain and excursions, Disposal, Consent in children and young people.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No period of validity was stated.

Inclusion: the previous version covered only children eligible under the NHS childhood programme, contradicting its own guidance summary and excluding the adults, healthcare workers, students and travellers a pharmacy service sees. Now any individual from 12 months without two documented doses. NHS-eligible children must be told the vaccine is free from their GP before any private supply.

Exclusion criteria, both arms: SmPC contraindications added: anaphylaxis to a previous MMR-containing vaccine, active untreated tuberculosis, blood dyscrasias, leukaemia, lymphoma or other malignancy of the haematopoietic or lymphatic system, family history of

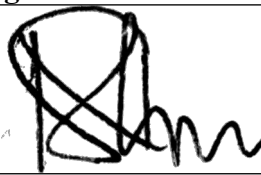
congenital or hereditary immunodeficiency unless immune competence demonstrated, and yellow fever or varicella vaccine in the previous 4 weeks.

Dose: two doses of 0.5 mL at least 4 weeks apart (3 months where both are given under 18 months), doses before the first birthday not counted. The previous text gave no interval.

Cautions: blood products or immunoglobulin in the previous 3 months handled per the SmPC and Green Book. Records: batch number, expiry, site, dose number and next dose date added.

Signed on behalf of Get Real Health

Version v004, 11 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		11 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		11 September 2026

Both authorising signatories reviewed this version together on 11 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.